



MONEY YOU'RE OWED

Medical Bill Dispute Kit

CLAIM WHAT'S RIGHTFULLY YOURS

The official process is always free. This kit doesn't sell you secrets — it's the done-for-you implementation: fill-in letters, word-for-word scripts, and trackers that save you the hours. Every method is built on official 2026 U.S. rules (DOT, IRS, CFPB, FDIC, state programs).

The Medical Bill Dispute Kit

Money You're Owed — practical tools for getting back the money that's rightfully yours.

The promise: Stop guessing. Fill in the blanks, send the right letter at the right time, and put the burden of proof back where it belongs — on the hospital and the insurer.

Read this first

The official process is always free. Every right in this kit — requesting an itemized bill, disputing an error, appealing an insurance denial, applying for hospital charity care, disputing a credit-report entry — is something you are entitled to do yourself, for \$0, without hiring anyone. No company can charge you to "unlock" these rights, and you should never pay a "finder" or a "negotiator" a percentage of money the law already lets you claim for free.

So what are you paying for? Time and certainty. This kit is the done-for-you implementation: eight ready-to-send letters with the correct law cited and the correct deadlines built in, a flowchart that tells you which letter to send when, and a tracker so nothing falls through the cracks. We did the drafting and the legal homework. You fill in your details and hit send.

This is not legal advice. It's a set of well-built templates grounded in current federal law (verified June 2026). Laws and your specific plan rules vary; for a complex or high-dollar dispute, or anything involving a lawsuit, talk to a licensed attorney or your state's free consumer-assistance program.

How to use this kit (the 60-second version)

1. **Always start by getting the itemized bill** (Letter 1). You cannot dispute charges you can't see. A "summary" bill is not enough.
2. **Compare** the itemized bill to your Explanation of Benefits (EOB) from your insurer and to any good-faith estimate you were given.
3. **Pick your situation** using the Decision Flowchart (file 09). It routes you to the exact letter.
4. **Send the letter** — fill in every [BRACKETED PLACEHOLDER], delete the instructions in italics, and send by a method that creates a paper trail (certified mail with return receipt, or the insurer's/hospital's secure portal with a saved confirmation).
5. **Log it** in the Dispute Tracker (10-dispute-tracker.md, with an importable 10-dispute-tracker.csv) the moment you send. Track the deadline by which they must respond.
6. **Never pay a bill you're actively disputing** unless you've decided to, and never pay before you've confirmed it's correct and that insurance has been applied.

Table of contents

#	Asset	Use it when...
00	This page — intro & how-to	Start here
01	Itemized-Bill Request	You got a bill but no line-by-line breakdown
02	Billing-Error / Overcharge Dispute	The itemized bill has wrong codes, duplicates, or charges for care you didn't get
03	Surprise / Balance-Bill Dispute (No Surprises Act)	You were billed extra by an out-of-network provider for an emergency or at an in-network facility
04	Internal Insurance Appeal	Your insurer denied or underpaid a claim
05	External Independent Review Request	Your internal appeal failed and you want an outside reviewer
06	Hospital Financial-Assistance / Charity-Care Request (501(r))	The bill is unaffordable and the hospital is a nonprofit
07	Settlement & Payment-Plan Offer	The charge is correct but you can't pay it in full
08	Medical-Debt Credit-Reporting Dispute (FCRA)	A medical collection is on your credit report and is wrong, paid, or shouldn't be there
09	Decision Flowchart	You're not sure which letter to send
10	Dispute Tracker (+ importable 10-dispute-tracker.csv)	Track every letter, deadline, and outcome

The laws this kit uses (plain-English, current as of June 2026)

You don't need to memorize these — each letter cites the right one. This is just so you know the ground is solid.

- **No Surprises Act (NSA)** — Federal law in effect since 2022. Bans "balance billing" (the gap between what an out-of-network provider charges and what your plan pays) for **emergency services** and for **out-of-network providers working at an in-network facility** (anesthesia, radiology, pathology, the assistant surgeon, etc.). Also gives the **uninsured/self-pay a written Good Faith Estimate** up front. *Used in Letter 3.*
- **IRS Section 501(r)** — Applies to **nonprofit (501(c)(3)) hospitals**. They must have a written **Financial Assistance Policy (FAP)**, cannot charge FAP-eligible patients more than the **Amounts Generally Billed (AGB)** to insured patients, and must give you a window (commonly **240 days from your first bill**) to apply before taking "extraordinary collection actions." *Used in Letter 6.*
- **ACA / ERISA appeal rights** — You get **at least 180 days** to file an **internal appeal** after a claim denial, and then **4 months** to request an **external independent review** by a neutral third party. Plans must decide internal appeals within set windows (generally 30 days pre-service, 60 days post-service; 72 hours if urgent). *Used in Letters 4 and 5.*
- **Fair Credit Reporting Act (FCRA)** — Your right to **dispute** inaccurate items on your credit report and force the bureau and the furnisher to investigate. *Used in Letter 8.*

Important reality check on medical debt and credit reports (June 2026): In January 2025 the CFPB finalized a rule that would have banned most medical debt from credit reports. **A federal court vacated that rule in July 2025**, so it is **not in effect**. This kit reflects that reality and does **not** claim medical debt is automatically off your report. What is still true (separate from that rule — these are voluntary changes the three major credit bureaus made in 2022–2023 and have kept): - **Paid** medical collections are removed. - Medical collections **under \$500** are not reported. - There is a **12-month grace period** before any unpaid medical collection can appear on your report. Letter 8 uses your durable FCRA dispute rights, which are unaffected by the court ruling.

Money You're Owed · Medical Bill Dispute Kit · Edition June 2026. Verify any deadline against your own denial letter, bill, or plan documents — your specific dates control.

Letter 1 — Itemized-Bill Request

When/how to use this: Send this the moment you receive a medical bill that shows only a lump sum, a "balance due," or vague department totals. You have the right to see every individual charge — the procedure, the supply, the medication, each with its billing code (CPT/HCPCS) and price. **Do this first, before disputing anything.** You cannot challenge a charge you can't see, and a surprising share of errors (duplicate charges, services never rendered, upcoding) only become visible on the itemized statement. Send by certified mail with return receipt, or through the provider's patient portal with a saved screenshot of the submission.

What law backs it: Billing transparency is part of standard hospital billing practice and, for nonprofit hospitals, is reinforced by the IRS Section 501(r) billing-and-collections requirements. You also need the itemized statement to exercise your insurance-appeal and No-Surprises-Act rights. There is no charge for an itemized bill — it is a routine request providers are expected to honor.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[Billing Department / Patient Financial Services] [Hospital or Provider Name] [Provider Street Address] [City, State, ZIP]

Re: Request for a fully itemized bill Patient name: [Patient Full Name] Date(s) of service: [MM/DD/YYYY] – [MM/DD/YYYY] Account / statement number: [Account #] Medical record number (if known): [MRN]

To the Billing Department:

I am writing to request a complete, itemized statement for the account referenced above. The bill I received shows a total balance but does not break down the individual charges, so I am unable to review it for accuracy.

Please send me an itemized statement that includes, for **every** charge:

1. The date of each service or item;
2. A plain-language description of the service, supply, or medication;
3. The corresponding billing code for each line (CPT, HCPCS, and/or revenue code), and the diagnosis (ICD-10) codes used;
4. The quantity and the per-unit charge;
5. The full charge amount for each line item;
6. Any payments, adjustments, insurance write-offs, or discounts already applied; and
7. The current balance you believe I owe after those adjustments.

If any portion of this account has been submitted to my insurer, please also confirm the date it was submitted and the claim number, so I can match the bill to my Explanation of Benefits.

I am requesting this information so I can verify the charges before making payment. **Please consider any amount I might owe to be in dispute until I have received and reviewed the itemized statement.** Kindly place a hold on any collection activity, late fees, or interest on this account until you have provided the requested itemization and I have had a reasonable opportunity to review it.

Please send the itemized statement to the address above within **30 days**. If you need anything from me to process this request, contact me at the phone number or email listed above.

Thank you for your prompt attention.

Sincerely,

[Your Signature] [Your Printed Name]

Enclosures (if applicable): copy of the summary bill received; copy of insurance card (front/back).

Quick notes before you send

- **Keep the original bill.** Send copies, never your only copy.
- **Match it to your EOB.** When the itemized bill arrives, lay it next to your insurer's Explanation of Benefits. Look for charges that were never submitted to insurance, services on dates you weren't there, and duplicate line items.
- **Next step:** If the itemized bill reveals errors → **Letter 2**. If it reveals an out-of-network surprise charge → **Letter 3**. If it's correct but unaffordable → **Letter 6** (nonprofit hospital) or **Letter 7** (payment plan).

Letter 2 — Billing-Error / Overcharge Dispute

When/how to use this: Send this after you've received the itemized bill (Letter 1) and spotted specific problems — duplicate charges, services or supplies you never received, the wrong quantity, a procedure code that doesn't match what was done ("upcoding"), a charge that should have been bundled, or a price that doesn't match a good-faith estimate or quoted rate. List each disputed item precisely. Send by certified mail with return receipt, or via the portal with a saved confirmation. **Do not pay the disputed amount while it's under review.**

What law backs it: This is a direct billing dispute with the provider. For nonprofit hospitals, IRS Section 501(r) requires reasonable efforts to resolve billing before extraordinary collection actions. If insurance was involved and the "error" is actually a coverage decision, use Letter 4 (insurance appeal) instead. Disputing your own bill costs nothing and does not require a third party.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[Billing Department / Patient Financial Services] [Hospital or Provider Name] [Provider Street Address] [City, State, ZIP]

Re: Dispute of specific billing errors — request for correction Patient name: [Patient Full Name] Date(s) of service: [MM/DD/YYYY] – [MM/DD/YYYY] Account / statement number: [Account #]

To the Billing Department:

I have reviewed the itemized statement for the account above against my records and my insurer's Explanation of Benefits. I dispute the following charges as incorrect and am requesting that you investigate and correct them:

#	Date	Charge / code as billed	Amount	Why it is disputed
1	[MM/DD/YYYY]	[description / CPT code]	[\$[amount]]	[e.g., "Duplicate of line 4 — same code, same date, billed twice"]
2	[MM/DD/YYYY]	[description / CPT code]	[\$[amount]]	[e.g., "Service never provided — I did not receive this medication"]
3	[MM/DD/YYYY]	[description / CPT code]	[\$[amount]]	[e.g., "Quantity billed as 3; medical record shows 1"]
4	[MM/DD/YYYY]	[description / CPT code]	[\$[amount]]	[e.g., "Code billed is a higher-level service than what was performed"]

Total amount in dispute: \$[total]

[Optional — include if relevant:] I was given a good-faith estimate / quote of **[\$[quoted amount]]** for this care on [date], which is attached. The billed amount exceeds that estimate without explanation, and I am asking you to reconcile the difference.

Please take the following steps:

1. **Investigate** each disputed line against the medical record and the actual services rendered;
2. **Correct or remove** any charge that is duplicated, was not provided, was billed in the wrong quantity, or was coded incorrectly;
3. **Re-bill my insurer**, if appropriate, for any charge that was not properly submitted; and
4. **Send me a corrected itemized statement** reflecting the outcome.

Until this dispute is resolved, please **place a hold on the disputed amount and on any collection activity, late fees, or interest** related to it. I am ready and willing to pay any amount that is confirmed to be accurate; I am only disputing the specific items listed above.

Please respond in writing within **30 days** to the address above. If you need records or documentation from me, contact me at the phone or email listed.

Sincerely,

[Your Signature] [Your Printed Name]

Enclosures: itemized statement (with disputed lines marked); copy of EOB; good-faith estimate or quote (if any).

Quick notes before you send

- **Be specific, not emotional.** "Line 12 duplicates line 4" wins; "this bill is outrageous" does not.
- **Mark up a copy** of the itemized bill (highlight the disputed lines, number them to match your table) and enclose it.
- **If they refuse or go silent past 30 days:** escalate to a supervisor in writing, and consider filing with your state attorney general or department of insurance/health.
- **Next step:** If the "error" turns out to be an insurance denial → **Letter 4**. If it's a true out-of-network surprise bill → **Letter 3**.

Letter 3 — Surprise / Balance-Bill Dispute (No Surprises Act)

When/how to use this: Send this when an out-of-network provider has billed you for **more than your in-network cost-sharing** in a situation the No Surprises Act protects. The classic cases: - **Emergency care** at any hospital or freestanding ER, in- or out-of-network. - **Out-of-network providers at an in-network facility** — the anesthesiologist, radiologist, pathologist, assistant surgeon, or other ancillary provider you didn't choose, during a visit to an in-network hospital or surgery center. - **Air ambulance** services.

In these cases the most you can be charged is the in-network cost-sharing amount (copay/coinsurance/deductible), and the provider may **not** "balance bill" you the difference. If you got this bill, send this letter to **both the provider and your insurer**. Send certified mail / portal with confirmation. Do not pay the balance-billed amount.

What law backs it: The **No Surprises Act** (in effect since January 1, 2022; federal regulations administered by CMS and the Departments of HHS, Labor, and Treasury). It bans balance billing for emergency services and for non-emergency services by out-of-network providers at in-network facilities, and bars providers from making you waive these protections for ancillary services. If you are **uninsured or self-pay**, the NSA also entitles you to a written **Good Faith Estimate**, and you can dispute a final bill that exceeds it by **\$400 or more** through the federal Patient-Provider Dispute Resolution process.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[Billing Department, Provider Name] **AND copy to:** [Insurance Company, Appeals/Member Services] [Provider Address] [Insurer Address]

Re: Improper balance bill — No Surprises Act protections apply Patient name: [Patient Full Name] Date(s) of service: [MM/DD/YYYY]
Provider account number: [Account #] **Insurance member ID / claim number:** [Member ID] / [Claim #] **Facility where care was provided:** [Facility Name] (in-network: [Yes/No])

To the Billing Department and to [Insurer]:

I received a bill from [Provider Name] for **\$[balance-billed amount]** above my in-network cost-sharing for services on [date]. I believe this is an improper balance bill that is prohibited by the **federal No Surprises Act**.

This care qualifies for No Surprises Act protection because (*check the one that applies and delete the others*):

- ☐ **It was emergency care.** I received emergency services, and the Act caps my responsibility at my plan's in-network cost-sharing regardless of whether the provider or facility was in-network.
- ☐ **It was an out-of-network provider at an in-network facility.** The facility, [Facility Name], is in my plan's network, but [Provider Name] (providing [anesthesia / radiology / pathology / assistant-surgery / other ancillary] services) was out-of-network. I did not knowingly choose this provider and did not give valid written consent to waive my balance-billing protections. For these ancillary services, such a waiver is not even permitted.
- ☐ **It was air ambulance service** furnished by an out-of-network provider.

Under the No Surprises Act, the most I can be required to pay is my **in-network cost-sharing amount** for these services. The balance-billed amount of **\$[balance-billed amount]** is not permitted.

I therefore request the following:

To [Provider Name]: 1. **Withdraw the balance bill** and rebill me for no more than my in-network cost-sharing; 2. **Cease any collection activity, late fees, and interest** on the disputed amount; and 3. If you believe you are entitled to additional payment, pursue it through the **federal Independent Dispute Resolution (IDR)** process with my insurer — **not** by billing me.

To [Insurer]: 1. **Process this claim under the No Surprises Act** and apply my in-network cost-sharing; and 2. Confirm in writing the correct in-network cost-sharing amount for this service.

[Use this paragraph **ONLY** if you are uninsured / self-pay:] I was uninsured/self-pay for this care. I was given a Good Faith Estimate of **\$[estimate amount]** on [date], and the final bill exceeds it by \$400 or more. I intend to use the federal **Patient-Provider Dispute Resolution** process and request that you hold collection on this account while that proceeds.

Please respond in writing within **30 days**. If you believe the No Surprises Act does not apply here, please explain specifically why, in writing.

For reference, patients can report suspected violations to the federal No Surprises Help Desk at **1-800-985-3059**.

Sincerely,

[Your Signature] [Your Printed Name]

Enclosures: copy of the bill; Explanation of Benefits; Good Faith Estimate (if uninsured); proof the facility was in-network (if applicable).

Quick notes before you send

- **Check the in-network status of the *facility*, not just the provider.** The protection for ancillary providers depends on the facility being in-network.
- **You did NOT waive your rights** unless you signed a specific NSA consent-to-waive form *and* it was a service where waiver is allowed (it is **never** allowed for emergency care or for ancillary providers like anesthesia/radiology/pathology).
- **Report it.** The No Surprises Help Desk (1-800-985-3059) takes complaints and is free.
- **Next step:** If your insurer still misprocesses the claim → **Letter 4** (internal appeal), then **Letter 5** (external review).

Letter 4 — Internal Insurance Appeal

When/how to use this: Send this when your health insurer **denied** a claim, **paid less than expected**, said a service was **not medically necessary**, called it **experimental/investigational**, or denied for a **coverage or network** reason. This is your **internal appeal** — the first formal step, handled inside the insurance company. You generally have **at least 180 days from the date on the denial notice (the EOB or denial letter)** to file. Send by certified mail and/or the insurer's portal; keep proof of the date. If the situation is urgent (delay would seriously jeopardize health), say so and request an **expedited** review.

What law backs it: The **Affordable Care Act (ACA)** and, for employer plans, **ERISA**, guarantee a full and fair internal appeal. ACA-compliant plans must give you **at least 180 days** to file an internal appeal, and must decide within set windows: generally **30 days** for services not yet received (pre-service), **60 days** for services already received (post-service), and **72 hours** for urgent/expedited cases. You also have the right to request, **free of charge**, the documents and criteria the insurer used to make its decision.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[Insurance Company Name] [Appeals Department] [Insurer Address]

Re: Internal appeal of claim denial / underpayment Member name: [Patient Full Name] Member ID: [Member ID #] Group number: [Group #]
Claim number(s): [Claim #] Date(s) of service: [MM/DD/YYYY] Date of denial notice being appealed: [MM/DD/YYYY]

To the Appeals Department:

I am formally appealing your decision to [deny / underpay] the claim referenced above. Please treat this as a request for a full internal appeal under my plan and applicable federal law (ACA/ERISA).

The reason you gave for the denial was: [quote the exact reason from the EOB / denial letter, e.g., "not medically necessary," "out-of-network," "service not covered," "needed prior authorization," "experimental/investigational"].

I am appealing because: (*use the points that apply; be specific and factual*)

- The service was **medically necessary**. [Brief explanation. Reference enclosed letter of medical necessity / records from Dr. [Name].]
- The denial appears to be a **coding or processing error**. [Explain — e.g., the claim was filed under the wrong code, or a prior authorization on file (auth # []) was not applied.]
- This care is **covered under my plan**. [Cite the plan provision / Summary of Benefits language if you have it.]
- [*If applicable*] No Surprises Act protections apply to this service, so my cost-sharing should be at the in-network level. [See enclosed.]
- [*If applicable*] The denial conflicts with the treating physician's recommendation, which is enclosed.

I request that you: 1. **Overturn the denial** and process/pay the claim; 2. Provide me, **free of charge**, with copies of all documents, records, internal guidelines, and medical-necessity criteria used to make this decision, and the name and credentials of the reviewer; and 3. Confirm your decision in writing, including my further appeal rights.

[*Include this paragraph ONLY for urgent cases:*] **This is a request for an EXPEDITED appeal.** Waiting the standard timeframe would seriously jeopardize the patient's health or ability to regain maximum function. Please decide within **72 hours** and contact me immediately at [phone].

Per federal law, please render your internal-appeal decision within the required timeframe — generally **30 days** for pre-service claims and **60 days** for post-service claims. If you uphold the denial, please provide a written explanation and confirm my right to request an **external independent review**.

Sincerely,

[Your Signature] [Your Printed Name]

Enclosures: copy of EOB / denial letter; letter of medical necessity from treating physician; relevant medical records; copy of prior authorization (if any); any supporting clinical literature.

Quick notes before you send

- **The strongest appeals come from the doctor.** Ask your physician's office for a **letter of medical necessity** — it dramatically raises your odds.
- **Quote their exact denial reason** and rebut *that* reason. Don't argue a point they didn't make.
- **File on time.** Use the date on the denial notice and count your 180 days. File early.
- **Save everything** — the date you mailed/submitted, the claim numbers, every reference number a rep gives you.
- **Next step:** If the internal appeal is denied → **Letter 5** (external review). You generally have **4 months** from the final internal denial.

Letter 5 — External Independent Review Request

When/how to use this: Send this **after your internal appeal (Letter 4) has been denied** — when the insurer upheld its decision and gave you a "final internal adverse determination." External review hands your case to an **independent third party** (an Independent Review Organization) that is **not** the insurer. Its decision is **binding** on the insurer. You generally have **4 months (120 days) from the date of the final internal denial** to request it. Send to the address/instructions on your final denial letter; if none, your state's department of insurance or the federal external-review process (often run by an HHS-contracted administrator) can route it. For urgent cases you can request an **expedited external review** — sometimes simultaneously with the internal appeal.

What law backs it: The **Affordable Care Act** guarantees external review for most plans. Decisions on denials based on **medical necessity, appropriateness, or experimental/investigational** status are sent to an Independent Review Organization. Standard external decisions are due within **45 days**; **expedited** decisions within **72 hours**. The reviewer's decision binds the insurer. Independent reviewers overturn a meaningful share of denials, so this step is worth taking.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[External Review Administrator / Insurer External-Review Unit / State DOI] (*use the contact named on your final internal-denial letter*) [Address]

Re: Request for external independent review **Member name:** [Patient Full Name] **Member ID:** [Member ID #] **Plan / insurer:** [Insurance Company Name] **Claim number(s):** [Claim #] **Date(s) of service:** [MM/DD/YYYY] **Date of FINAL internal denial:** [MM/DD/YYYY]

To the External Review Administrator:

I am requesting an **external independent review** of the final adverse determination issued by [Insurance Company Name] on [date]. I have completed the insurer's internal appeal process, and the denial was upheld. I am submitting this request within the **four-month** window.

Service/treatment at issue: [describe — e.g., "MRI of lumbar spine," "30-day inpatient rehabilitation," "[drug name]"]

The insurer's final reason for denial was: [quote exactly — e.g., "not medically necessary," "experimental/investigational"].

I am requesting external review because I believe the denial is wrong for the following reasons: (*use what applies*) - The treating physician has determined this care is medically necessary [see enclosed letter of medical necessity]; - The service is the recognized standard of care for my condition [see enclosed clinical documentation]; - The "experimental/investigational" label is incorrect because [explanation / cite guideline or FDA approval]; - The insurer did not adequately consider the records I provided in the internal appeal.

I am enclosing the following for the independent reviewer: 1. The insurer's final internal-denial letter; 2. The original Explanation of Benefits / denial; 3. My internal appeal letter and all supporting documents I previously submitted; 4. A letter of medical necessity from my treating physician, Dr. [Name]; 5. Relevant medical records and any supporting clinical literature.

I authorize the release of my relevant medical records to the Independent Review Organization for the purpose of this review.

[*Include ONLY for urgent cases:*] **This is a request for an EXPEDITED external review.** A delay would seriously jeopardize the patient's life, health, or ability to regain maximum function. Please decide within **72 hours** and contact me immediately at [phone].

Please confirm receipt and provide the assigned reviewer and expected decision date. I understand the standard decision is due within **45 days** and that the decision will be **binding** on the insurer.

Sincerely,

[Your Signature] [Your Printed Name]

Enclosures: final internal-denial letter; original EOB/denial; prior internal appeal packet; physician letter of medical necessity; medical records; clinical literature; signed records-release authorization.

Quick notes before you send

- **Read your final denial letter for the exact route.** It must tell you how to request external review and the deadline. Some states run their own process; others use the federal HHS-administered process. Follow whichever your letter names.
- **Resend your best evidence.** The external reviewer reads what you give them — include the physician letter and records again, even if the insurer already had them.
- **It's free** to you. Insurers fund the external-review process; you don't pay the reviewer.
- **Binding decision.** If the IRO sides with you, the insurer must cover the service. If it doesn't, ask about further options (state regulator complaint, or legal advice for ERISA plans).
- **Next step:** If you still owe a balance the insurer won't cover and the bill is unaffordable → **Letter 6** (nonprofit charity care) or **Letter 7** (payment plan/settlement).

Letter 6 — Hospital Financial-Assistance / Charity-Care Request (501(r))

When/how to use this: Send this when a hospital bill is unaffordable and the hospital is a **nonprofit (501(c)(3))** — which includes most U.S. hospitals, including many large systems and most religious and university hospitals. Nonprofit hospitals are **required by federal law** to have a written Financial Assistance Policy (FAP) that can reduce or **eliminate** your bill based on income. This letter requests the FAP and the application, and asks them to **pause collections** while you apply. Send early — ideally as soon as you get a bill you can't afford — but you generally have a long window (commonly **240 days from your first bill**) to apply. Certified mail / portal with confirmation.

What law backs it: **IRS Section 501(r)**, which all 501(c)(3) hospitals must follow to keep their tax-exempt status. It requires them to (1) maintain and widely publicize a written **Financial Assistance Policy**; (2) limit charges to FAP-eligible patients to no more than the **Amounts Generally Billed (AGB)** — i.e., roughly what insured patients pay, not inflated "chargemaster" rates; and (3) make **reasonable efforts to determine FAP eligibility before extraordinary collection actions** (like sending you to collections, suing, or reporting to credit bureaus). Many hospitals must accept applications for **at least 240 days** from the first post-discharge bill. Applying for charity care is always free.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[Hospital Name] [Financial Assistance / Patient Financial Services Department] [Hospital Address]

Re: Request for financial assistance under your Financial Assistance Policy (IRS §501(r)) Patient name: [Patient Full Name] Date(s) of service: [MM/DD/YYYY] Account / statement number: [Account #] Total balance billed: \$[amount]

To the Financial Assistance Department:

I am requesting financial assistance for the account above. I understand that, as a nonprofit hospital, [Hospital Name] maintains a written **Financial Assistance Policy (FAP)** under IRS Section 501(r), and I would like to apply.

Please send me, as soon as possible: 1. A **plain-language summary of your Financial Assistance Policy** and the **full FAP**; 2. The **financial assistance application form** and a list of the documents required to apply; 3. Your **FAP-eligibility income criteria** (e.g., the Federal Poverty Level percentages you use for free vs. discounted care); and 4. An explanation of the **Amounts Generally Billed (AGB)** for the services on my account, since under §501(r) I cannot be charged more than the AGB if I am FAP-eligible.

My situation: [Briefly describe — e.g., household size, income, recent job loss, other large medical bills, that the balance exceeds what you can reasonably pay. Keep it factual.]

I request that, while my application is pending, you: - **Hold all collection activity**, including sending this account to a collection agency, reporting it to credit bureaus, charging interest or late fees, or any extraordinary collection action; and - Confirm in writing the deadline by which I must submit my completed application (I understand this is commonly at least **240 days from my first bill**).

If I have already paid any amount on this account that exceeds what I would owe as a FAP-eligible patient, I request a **refund of the difference**, consistent with your policy.

Please send the policy and application to the address above and confirm receipt of this request. If you need additional information from me to begin the eligibility determination, contact me at [phone] or [email].

Thank you for your help.

Sincerely,

[Your Signature] [Your Printed Name]

Enclosures (if ready): proof of income (recent pay stubs / benefit letter / prior-year tax return); proof of household size; documentation of hardship.

Quick notes before you send

- **Confirm the hospital is nonprofit.** Search the hospital name + "Financial Assistance Policy" or "501(r)" — almost every nonprofit posts it. If it's a for-profit hospital, 501(r) doesn't apply, but most still have a charity/discount program — ask anyway and use **Letter 7** to negotiate.
- **You can qualify even with insurance.** FAP eligibility is about income and the size of the bill, not whether you're insured.
- **Apply even if it's already in collections.** The 240-day window and the "reasonable efforts" rule can require them to pull it back and review you.
- **Watch the AGB cap.** If they bill you full "sticker" charges after you qualify, point to §501(r)(5) — they can't charge FAP-eligible patients more than AGB.
- **Next step:** If you don't qualify for full charity care but still can't pay in a lump sum → **Letter 7** (settlement / payment plan).

Letter 7 — Settlement & Payment-Plan Offer

When/how to use this: Send this when the charge is **confirmed correct** (you've already done Letters 1–2 and any insurance appeal), but you **can't pay it in full**. You have two plays here, and the letter covers both: (A) offer a **lump-sum settlement** for less than the full balance in exchange for marking the account paid-in-full, or (B) request an **interest-free monthly payment plan** you can actually afford. Hospitals and collectors routinely accept both, because partial payment now beats chasing you later. **Get any agreement in writing before you pay a cent.** Send certified mail / portal with confirmation.

What law backs it: This is a negotiation, not a statutory right, so there's no single law to cite — and that's fine. Use it *after* you've exhausted the rights-based letters. One protection to know: if a **debt collector** (not the original hospital) holds the account, the **Fair Debt Collection Practices Act (FDCPA)** governs how they may contact you, and you can request **written validation of the debt**. If the hospital is nonprofit, prefer **Letter 6** first — charity care can wipe out far more than a settlement.

[Your Full Name] [Your Street Address] [City, State, ZIP] [Your Phone] · [Your Email] [Date]

[Hospital / Provider / Collection Agency Name] [Billing or Account-Resolution Department] [Address]

Re: Offer to resolve account — [settlement / payment plan] **Patient name:** [Patient Full Name] **Account / reference number:** [Account #]
Current balance: \$[balance]

To the Billing / Account-Resolution Department:

I want to resolve the account above. I have confirmed the charges are correct, and I am committed to paying what I genuinely can. I cannot pay the full balance in a lump sum, so I am making the following good-faith offer.

Choose and keep ONE of the two options below; delete the other.

OPTION A — Lump-sum settlement

I am prepared to pay **\$[offer amount]** (approximately []% of the balance) as **payment in full** to settle and close this account. This is the most I am able to pay at one time. In exchange, I ask that you agree in writing to: 1. Accept **\$[offer amount]** as **full and final satisfaction** of the account; 2. **Not pursue** the remaining balance, sell it, or refer it to collections; and 3. Report the account as **"paid in full" / "settled, \$0 balance,"** and, if it has been reported to any credit bureau, request its **deletion or update to reflect a zero balance**.

Upon receiving your written agreement to these terms, I will send payment within [10] business days by [method].

OPTION B — Interest-free monthly payment plan

I can reliably pay **\$[monthly amount] per month** until the balance is satisfied. I request a **payment plan with no interest and no fees**, and ask that you agree in writing to: 1. Accept monthly payments of **\$[monthly amount]** beginning [date]; 2. **Charge no interest, late fees, or penalties** while I make agreed payments on time; 3. **Not refer the account to collections or report it adversely** to credit bureaus while the plan is in good standing; and 4. Send me a written confirmation of these terms and a simple receipt or running balance.

For both options: *[If a collection agency holds this debt, include:]* Please also provide **written validation of this debt** (the amount, the original creditor, and that it is mine) before I make any payment, as is my right under the Fair Debt Collection Practices Act.

Please respond in writing within **30 days**. I am acting in good faith and want to close this out fairly; I just need terms I can actually meet. Do not consider any payment I make to be an admission regarding any portion of the bill I have previously disputed.

Sincerely,

[Your Signature] [Your Printed Name]

Quick notes before you send

- **Get it in writing FIRST.** Never pay on a verbal promise. A "paid in full" agreement is worthless if it's not on paper before your money moves.
- **Start your settlement offer low** (e.g., 25–40% of the balance) — you can come up; you can't come down.
- **Try charity care first** at nonprofit hospitals (**Letter 6**). A settlement at 40% is a worse deal than a charity-care write-off at 100%.
- **Don't let a payment "reset the clock."** On old debts, making a payment can sometimes restart the statute of limitations in some states — ask before paying on an old account.
- **Match the plan to reality.** Offer a monthly amount you can sustain for the full term. A blown payment plan is worse than a smaller one you keep.

Letter 8 — Medical-Debt Credit-Reporting Dispute

When/how to use this: Send this when a medical debt is showing up on your credit report and it shouldn't be — because it's **wrong, not yours, already paid, under \$500, less than a year old, or you never owed it** (for example, it was actually a No Surprises Act violation or your insurer should have paid it). You send this to the **credit bureaus** (Equifax, Experian, TransUnion) that are reporting the item, and you can also send a copy to the **collector or hospital** that furnished it. Send certified mail with return receipt, or use each bureau's online dispute portal and save the confirmation. Dispute with **every** bureau that shows the item — they're separate companies.

What law backs it: Two things, and it's important to understand the difference:

1. **The Fair Credit Reporting Act (FCRA), 15 U.S.C. §1681i and §1681s-2.** This is your permanent, federal right to dispute *anything inaccurate or unverifiable* on your credit report. Once you dispute, the bureau must investigate — generally **within 30 days** (up to 45 in some cases) — and **delete** any item it cannot verify. The furnisher (the hospital or collector) must investigate too and correct or remove what's wrong. These rights apply to all debts, medical or not.
2. **The voluntary credit-bureau medical-debt policy** (Equifax, Experian, and TransUnion, phased in 2022–2023, still in effect in 2026). Under it: **paid medical collections are removed**; medical collections are **not reported until at least one year (365 days)** after they go to collections; and medical collections with an original balance **under \$500 are not reported at all**. If a medical item violates one of these policies, you can dispute it on that basis.

The reality check that keeps this kit honest: You may have read that "medical debt is being removed from all credit reports." In **January 2025** the CFPB finalized a rule that would have done exactly that. **A federal court vacated that rule on July 11, 2025 (Cornerstone Credit Union League v. CFPB, E.D. Tex.), and it is NOT in force in 2026.** So a medical collection of \$500 or more that's over a year old **can still legally appear on your credit report** federally. Don't dispute it by citing the CFPB rule — that argument will fail. Dispute it on the grounds that actually work: it's inaccurate, it's paid, it's under \$500, it's too new, or your state bans it (several states do — check yours). This letter is built around the grounds that work.

[Your Full Name] [Your Mailing Address] [City, State, ZIP] [Date of Birth] · [Last 4 of SSN]

[Date]

[Credit Bureau Name — send a separate copy to each that reports the item] Equifax: P.O. Box 740256, Atlanta, GA 30374 Experian: P.O. Box 4500, Allen, TX 75013 TransUnion: P.O. Box 2000, Chester, PA 19016

Re: Dispute of inaccurate medical collection — request for investigation and deletion Reported item: [Collector/Furnisher Name], account/reference number [Account #] **Amount shown:** \$[Amount] **Date reported / date of first delinquency:** [Date shown on report]

To Whom It May Concern:

I am disputing the medical collection account identified above, which appears on my credit report. Under the **Fair Credit Reporting Act, 15 U.S.C. §1681i**, I request that you conduct a reasonable investigation of this item and **delete it** if it cannot be verified as accurate and reportable.

This item is inaccurate or improperly reported for the following reason(s) (*check all that apply and delete the rest*):

- ☐ **It is not mine / I do not recognize this debt.** I have no record of owing this amount to this provider or collector.
- ☐ **It is already paid.** This medical collection has been paid in full as of [date]. Under the credit bureaus' own medical-debt reporting policy, **paid medical collections are not reported**. Proof of payment is enclosed.
- ☐ **The original balance is under \$500.** The original amount of this medical collection was **less than \$500**, which under the credit bureaus' medical-debt reporting policy is **not reported at all**.
- ☐ **It is less than one year old.** This medical debt was placed in collections on [date], which is **less than 365 days ago**. Under the bureaus' policy, medical collections are not reported until at least one year after they go to collections.
- ☐ **The amount is wrong.** The correct balance is \$[correct amount], not \$[amount shown]. (*Attach the corrected/itemized bill.*)
- ☐ **I never owed this — it should have been covered.** This charge was an improper bill (for example, a balance bill prohibited by the No Surprises Act, or a service my insurer was required to pay). It was sent to collections in error. (*Attach supporting documents — EOB, insurer letter, etc.*)
- ☐ **It is being disputed with the provider.** I have an open billing dispute or insurance appeal on this charge, and it should not have been sent to collections or reported while unresolved.

I request that you:

1. Conduct a reasonable investigation of this item as required by **15 U.S.C. §1681i**;
2. Forward this dispute and all enclosed documents to the furnisher;
3. **Delete the item** if it cannot be verified as accurate and reportable; and
4. Send me the written results of your investigation and, if any change is made, a free updated copy of my credit report.

I am enclosing copies (not originals) of supporting documents. Please complete your investigation within the **30 days** required by the FCRA.

Sincerely,

[Your Signature] [Your Printed Name] [Date]

Enclosures: copy of the credit report with the item circled; proof of payment (if paid); itemized bill (if amount is wrong); EOB / insurer correspondence (if it should have been covered); copy of a government-issued ID and a utility bill (bureaus often require these to verify identity).

After you send

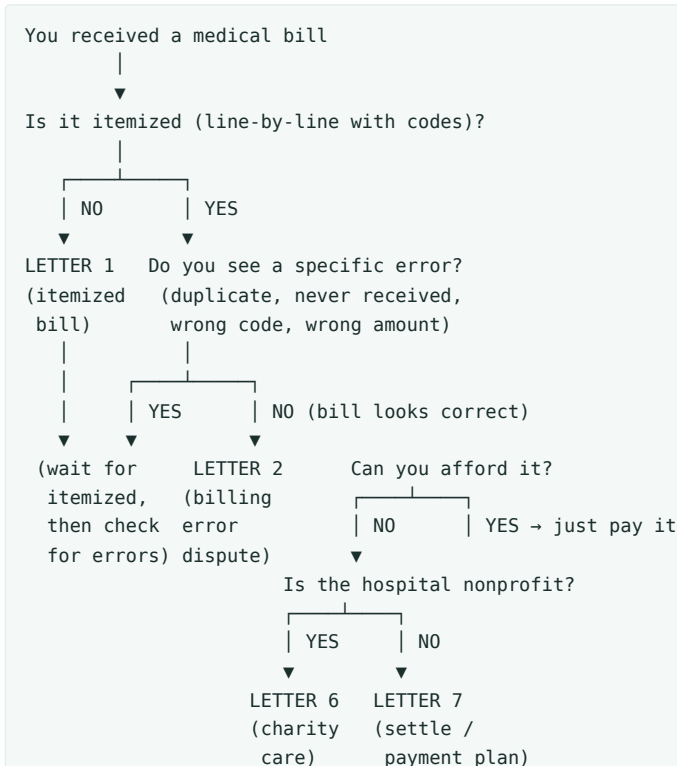
- **The bureau has ~30 days** (up to 45 if you send more information mid-investigation) to respond. If they verify it and you still believe it's wrong, you can: (a) add a **100-word statement of dispute** to your file, (b) file a complaint with the **CFPB** (consumerfinance.gov/complaint) or your state attorney general, or (c) consult an attorney — FCRA violations can entitle you to damages.
- **Also dispute with the furnisher.** Sending a copy to the collector or hospital triggers their duties under **§1681s-2**.
- **If the underlying bill is the real problem**, fix that first or alongside this: an itemized-bill request (Letter 1), an error dispute (Letter 2), a No Surprises Act dispute (Letter 3), or charity care (Letter 6) can make the debt itself disappear, which is the cleanest way to get it off your credit.
- **Check your state law.** Several states (including CA, CO, CT, IL, MD, MN, NJ, NY, RI, VA, others) restrict or ban medical-debt credit reporting beyond the federal floor. If you live in one, say so in your dispute and cite the state law.

Decision Flowchart — Which Letter, When?

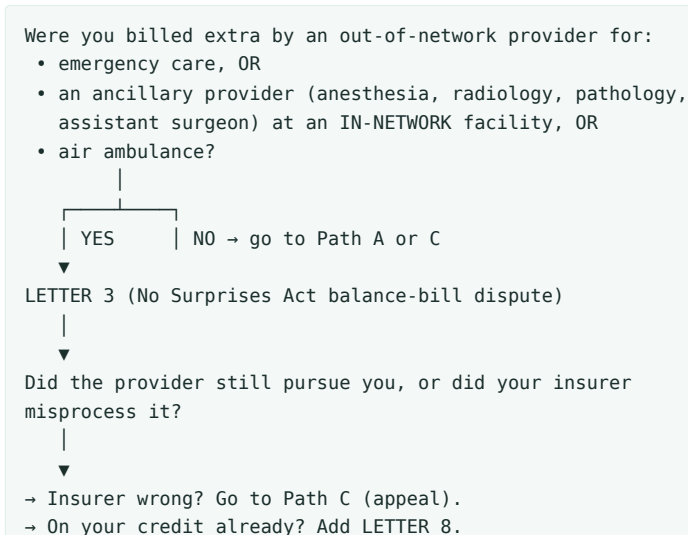
How to use this page: Start at the top. Follow the questions down. Each path ends at the letter (or letters) to send. One situation can need more than one letter — that's normal and noted below.

Start here: What just happened?

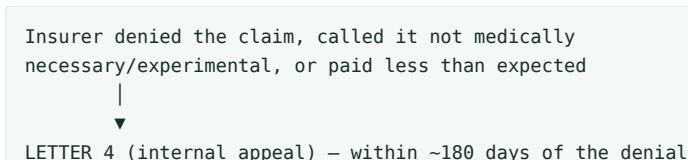
Path A — "I got a bill I want to check or think is wrong"



Path B — "An out-of-network or surprise charge"

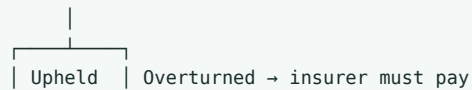


Path C — "My insurance denied or underpaid a claim"





LETTER 5 (external independent review)
– within ~4 months (120 days) of the final internal denial

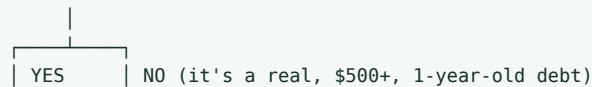


Reviewer's decision is binding on the insurer.
If you still owe a real balance you can't afford → Path A
(charity care / settlement).

Path D — "It's already on my credit report"

A medical collection is on your credit report

Is it wrong, paid, under \$500, or less than 1 year old?



LETTER 8 (credit dispute) The federal CFPB rule that would have removed it was VACATED in 2025 – it can legally stay. Best move: kill the underlying debt (Letters 1, 2, 3, or 6) or settle it (Letter 7), THEN dispute the entry (Letter 8). Also check your STATE law – several states ban it.

Common combinations (real situations need more than one letter)

Situation	Letters to send, in order
Big hospital bill, no detail, can't afford it, nonprofit hospital	1 → 6 (→ 7 if charity care isn't enough)
Surprise ER anesthesia bill	3 (→ 4/5 if insurer misprocesses → 8 if it hit credit)
Insurer denied a needed surgery	4 → 5
Bill is correct but huge	6 (if nonprofit) or 7
Old paid medical collection still on credit	8 (cite "paid medical collections aren't reported")
Collection under \$500 on credit	8 (cite the under-\$500 policy)
Wrong charge that already went to collections and to credit	1 → 2 → 8

The three deadlines you cannot miss

Action	Deadline	Letter
Internal insurance appeal	~180 days from the denial notice	Letter 4
External review	~4 months (120 days) from the final internal denial	Letter 5
Charity-care application	commonly 240 days from your first bill	Letter 6

Log every bill's deadlines in the Dispute Tracker the day you receive it.

Dispute Tracker

How to use this: One row per dispute. Fill it in the day a bill arrives so you never miss a deadline. The importable spreadsheet version is **10-dispute-tracker.csv** — open it in Excel, Google Sheets, or Numbers (File → Import / Open).

Status options: To send · Sent · Awaiting reply · Replied · Escalated · Resolved — won · Resolved — partial · Closed

#	Provider / Insurer	Patient	Date of service	Account / Claim #	Issue (short)	Letter(s) used	Disputed amount	Date sent	Sent how (mail/portal)	Deadline	Their reply date	Outcome	Amount saved	Next step / notes
1														
2														
3														
4														
5														
6														
7														
8														

Worked example (delete or keep as a guide)

#	Provider / Insurer	Patient	Date of service	Account / Claim #	Issue (short)	Letter(s) used	Disputed amount	Date sent	Sent how	Deadline	Their reply date	Outcome	Amount saved	Notes
Ex	Mercy General Hospital	Jane Doe	03/14/2026	ACCT 88421	Summary bill, no detail	Letter 1	n/a	03/20/2026	Certified mail	—	03/29/2026	Replied – itemized bill received	—	Final bill received
Ex	Mercy General Hospital	Jane Doe	03/14/2026	ACCT 88421	Duplicate \$1,200 charge	Letter 2	\$1,200	04/01/2026	Portal + saved PDF	—	04/15/2026	Resolved – won	\$1,200	Credit balance restored

Reminders to set in your phone calendar

- **Internal appeal:** due ~**180 days** after a denial — set a reminder for day 150.
- **External review:** due ~**4 months** after the final internal denial — set a reminder for week 14.
- **Charity care:** apply early; the window is often **240 days** from the first bill — don't wait for it.
- **Credit dispute:** the bureau owes you a result in ~**30 days** — set a day-35 reminder to follow up if you've heard nothing.

10 Dispute Tracker (importable spreadsheet)

This table is also provided as an editable .csv you can import into Google Sheets, Excel, or Notion.

#	Provider / Insurer	Patient	Date of Service	Account or Claim #	Issue (short)	Letter(s) Used	Disputed Amount	Date Sent	Sent How (mail/portal)	Deadline	Their Reply Date	Outcome	Amount Saved
EX	Mercy General Hospital	Jane Doe	03/14/2026	ACCT 88421	Summary bill - no detail	Letter 1		03/20/2026	Certified mail		03/29/2026	Replied - itemized bill received	
EX	Mercy General Hospital	Jane Doe	03/14/2026	ACCT 88421	Duplicate \$1200 charge	Letter 2	1200	04/01/2026	Portal + saved PDF		04/15/2026	Resolved - won	1200
1													
2													
3													
4													
5													
6													
7													
8													
9													
10													